



Placement and Residential Licensing Unit

P.O. Box 1437, Slot S140, Little Rock, AR 72203-1437

P: 501.682.8590 F: 501.683.6060 TDD: 501.682.1550

Notice of Serious Incident

Case Number: 030065

Date of Incident: 5/23/2025

Date Received: 5/27/2025

Facility Name: Perimeter Behavioral of Forrest City

Facility Number: 142

Incident Type: Licensing

Report Description: Resident, [REDACTED] was placed into a PRT due to Resident punching walls in his bedroom. No emergency medications administered. Respirations even and non-labored. Upon assessment, Resident noted with bruising to left hand and wrist. Resident stated to Nurse that Staff shoved him in his room and his wrist went backwards on his wall. Resident unable to bend wrist and hand. Left wrist and hand immobilized, evaluated, with ice pack applied. Resident sent to [REDACTED]

[REDACTED] Resident placed on assault precautions. Staff member named as alleged offender suspended pending investigation. Resident, witnessing peer, and alleged offended Staff statements collected. [REDACTED]

[REDACTED] Camera footage reviewed by Safety team. Resident returned to the facility at 1220 (12:20 am) from [REDACTED]

[REDACTED] No distress noted with complaint of pain to left upper extremity. [REDACTED]. Respirations even and non-labored. Resident noted with splint to upper left extremity, able to move distal fingers. New orders to follow up with [REDACTED] and [REDACTED] per Physician's orders. Resident assisted to bedroom by MHT staff. Resident is currently on Line of Sight while splint is in use.

Interim Action Narrative:

[REDACTED] **Narrative:**

Licensing Narrative: Program Coordinator received a phone call from an entity regarding this complaint. Per entity, they were not able to obtain requested documentation from the facility. Entity was informed that licensing was waiting for the complaint to be submitted and would inform the entity once received. 5/28/2025, licensing informed entity that complaint has been received 5.28.25- emailed facility inquiring name of suspended staff and for facility to retain footage facility replied: With the new information presented today, we are moving forward with suspending [REDACTED] today. [REDACTED]. Footage is retained. Program Coordinator spoke with facility via telephone and explained that the staff member in question was named in this complaint and that licensing needed to know is employment status. Facility reported that staff member would be suspended as of today. Program Coordinator spoke with an entity via telephone regarding documentation received. Entity was informed that documentation had not been received as of today. 5/29/2025, Program Coordinator forwarded documentation received to the entity. Program Coordinator spoke with the entity via telephone who informed Licensing of more information regarding this complaint. 5.29.25- Uploaded Facility provided intervention packet, body check, and assessment. Licensing Specialist noted reason for restraint was imminent threat of self-harm, but staff notes state resident was slamming doors and not willing to obey staff and stop. Resident was in the restraint from 1911 to 1915, restraint packet was signed at 1914. Body Assessment shows scratches on inner thigh/groin area as well as markings and bruising on his upper arms and wrists. There was also no "Supervisor on Duty" signature on body assessment paperwork. Resident states he does not feel safe due to staff who put him in restraint being "aggressive both physically and verbally." 5/30/2025, licensing received documentation from an entity. Documentation uploaded and will be reviewed by the Licensing Specialist. Management reviewed provided video. 05/30/2025 Licensing Specialist visited the facility viewed camera footage and met with residents regarding Licensing concerns. 6.2.25- Licensing Specialist reviewed footage and uploaded narrative into notes. 6.2.25- Licensing Specialist called [REDACTED] and left voicemail. Called [REDACTED] and uploaded interview narrative into notes. 6.3.25- Specialist called [REDACTED] again and left voicemail, will also be sending email. Uploaded interview narrative. 6.6.25- uploaded PRT Certifications for staff as well as personnel file paperwork

DEPARTMENT OF HEALTH AND HUMAN SERVICES
CENTERS FOR MEDICARE & MEDICAID SERVICES

Approved 6/30/25 BBM

PRINTED: 06/20/2025
FORM APPROVED
OMB NO. 0938-0391

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: 04L115	(X2) MULTIPLE CONSTRUCTION A. BUILDING _____ B. WING _____		(X3) DATE SURVEY COMPLETED C 06/06/2025
NAME OF PROVIDER OR SUPPLIER WOODRIDGE OF FORREST CITY, LLC			STREET ADDRESS, CITY, STATE, ZIP CODE 1521 ALBERT ST FORREST CITY, AR 72335		
(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)		(X5) COMPLETION DATE
N 000	Initial Comments Note: The CMS-2567 (Statement of Deficiencies) is an official, legal document. All information must remain unchanged except for entering the plan of correction, correction dates, and the signature space. Any discrepancy in the original deficiency citation(s) will be reported to the Dallas Regional Office (RO) for referral to the Office of the Inspector General (OIG) for possible fraud. If information is inadvertently changed by the provider/supplier, the State Survey Agency (SA) should be notified immediately. Complaint #N/A (AR00036313) was in compliance. Complaint #N/A (AR00038422) was in compliance. Complaint #N/A (AR00038874) was not in compliance with deficiencies cited at N128 and N142 The facility was not in compliance with §483, Subpart G - Conditions of Participation for Psychiatric Residential Treatment Center.	N 000			
N 128	PROTECTION OF RESIDENTS CFR(s): 483.356(a)(3) Restraint or seclusion must not result in harm or injury to the resident and must be used only- This ELEMENT is not met as evidenced by: Based on observations, interviews, and record and video review, the facility failed to prevent injury during an Emergency Safety Intervention (ESI) for one (Client #3) of three clients reviewed for injuries. The findings include:	N 128	The Resident affected was assessed by Nursing and an Order was received by the Physician for further evaluation of Resident at local Medical center. Staff involved were suspended immediately and remain suspended pending investigation results. Results from internal investigation yield one Staff member not returning. The likelihood for this deficiency affecting other Residents being affected was greatly reduced and minimized due to the organization's response. The Chief Executive Officer oversees the implementation of this corrective action plan. A mandated facility-wide retraining for all Mental Health Technicians, Nurses, Dietary Staff, Maintenance Staff, Clinical staff, and Administrative staff on verbal de-escalation and proper physical interventions was conducted by the Handle with Care Instructor June 18 th 2025. Retraining included modeling verbal de-escalation, proper physical intervention, and scenario based participation. All attendees signed acknowledgement and attestation of training on proper Handle with Care technique. The Director of Quality and Risk Management is the custodian of the training log. Training records will be audited weekly by the Quality Risk Director until 100% training compliance is reached for existing staff. This training is already part of the new hire orientation process. The ongoing standard six-month		

		Handle with Care refreshers will also continue as scheduled, conducted by the Handle with Care Instructor and tracked by the Director of Quality and Risk Management. Compliance rate and upcoming trainings will be reviewed in the monthly Committee of the Whole meeting and reported to the Governing Board for oversight. For Staff who could not attend the training, individual training will be provided, documented, and attestations will be signed. All training will be completed by July 6th, 2025.	
LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE		TITLE	(X6) DATE

Any deficiency statement ending with an asterisk (*) denotes a deficiency which the institution may be excused from correcting providing it is determined that other safeguards provide sufficient protection to the patients . (See instructions.) Except for nursing homes, the findings stated above are disclosable 90 days following the date of survey whether or not a plan of correction is provided. For nursing homes, the above findings and plans of correction are disclosable 14 days following the date these documents are made available to the facility. If deficiencies are cited, an approved plan of correction is requisite to continued program participation.

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N 128	Continued From page 1 Review of Client #3's " Master Treatment Plan " dated 05/06/2025, revealed Client #3 had diagnoses which included major depressive disorder which was recurrent and severe without psychosis, generalized anxiety disorder, history of post-traumatic stress disorder, and history of trauma. Review of an ESI dated 05/23/25 and completed by Registered Nurse (RN) #1, revealed that Client #3 was placed in a physical restraint at 7:11 PM and was released at 7:15 PM, due to the resident being out of control and uncooperative with staff. ESI documentation included check marks under the section that listed interventions attempted before restraints were implemented as, emphasized need for self-control, attempted to redirect, and allowed patient other ways to vent anger. Review of the " Client Debrief Form" contained in the ESI packet revealed Client #3 reported that staff gave no warnings before the restraint was initiated. While in restraint the client reported that staff "just broke my arm." Review of a " Body Check Form " contained in the ESI packet under the comment section dated 05/23/2025 at 7:17 PM, signed by RN #1, revealed that per staff, Client #3 was placed in a personal restraint due to being uncooperative with staff and slamming the bedroom door in another resident's face. Client #3 was sent to the emergency department for an X-ray of the left wrist. A body assessment was completed by RN #1, that included a body diagram, which indicated a bruise on Client #3's upper area of both arms, and bruising on the left hand and wrist. The ESI	N 128			

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N 128	Continued From page 2 included Client #3 ' s complaints of pain and inability to move their left hand. Review of a " Nurse's Note " dated 05/23/2025 at 7:16 PM, revealed Client #3 reported that Mental Health Technician (MHT) #1 shoved Client #3 inside the bedroom causing the client's wrist and hand to go backwards. Client #3 was unable to move the upper left extremity, and bruising was noted to the entire wrist and hand. Review of an additional " Nurse's Note " timed 7:30 PM, indicated the client was transferred to the emergency department for evaluation and treatment. Both nurse ' s notes were signed by RN #1. Review of a " Nurse's Note " signed by RN #1, dated 05/24/2025 at 12:20 AM, indicated Client #3 returned from the hospital with a splint to the upper left extremity. Review of a handwritten witness statement by MHT #1, dated 05/23/2025, contained within the " Facility Investigation Packet " revealed Client #3 had called MHT #1 a derogatory name after she stepped on a bug that Client #3 was trying to put in a "container and save." Client #3 then went into the bedroom and slammed the door and was banging their hands on the clothes shelf. MHT #1 reported she told Client #3 to come out of the bedroom, but Client #3 was " resisting staff Client #3 finally came out and was placed in a primary restraint technique (PRT). Client #3 stated their wrist was hurting but staff had their arms and put them in a PRT with one hand so the client wouldn't say the staff was holding them too tight. " Review of a " witness statement, " of the incident transcribed and written by the Quality and Risk	N 128			

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N 128	Continued From page 3 Manager and signed by the resident on 05/26/2025, revealed Client #3 reported finding a bug then MHT #1 stepped on it and both Client #3 and MHT #1 began calling each other derogatory names. Client #3 went into their bedroom and slammed the door, then MHT #1 came into the room, slammed the door, screamed, "Don't slam this [expletive] door!" Client #3 stated, "I won't slam this [expletive] door in a sassy way." The client reported being shoved into the "cubby" by MHT #1, and then being put into a PRT and taken out into the hallway. Client #3 stated they asked MHT #1, "Could you release the pressure on my left arm because I think you broke it." Review of video footage dated 05/23/2025, time stamped beginning at 7:11:47 PM and ending at 7:16:22 PM, revealed the following events: At 7:11:49 Client #1 was seen noticing something on the floor by the double doors, turned and went quickly back to their bedroom, returned five seconds later with a red container and went back to the door and was seen bending down. MHT #1 arrived at the door and stepped in front of Client #3. Client #3 went back to their room and shut the door at 7:12:15. One second later, MHT #1 was seen aggressively pushing past Client #4 who was standing in the doorway. Client #4 continued to look into the open doorway and then the door closed again at 7:12:26 PM. One second later, Client #3 was seen coming out of the room with MHT #1 holding the client's left upper extremity around the biceps area with her left hand. MHT #1's arm was hooked under the client's right armpit which resulted in the right arm being held behind the client's back. MHT #1 then forcefully put Client #3 against the hallway wall pulling the client's elbows at an approximate 90-degree	N 128			

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N 128	Continued From page 4 angle to the client's body up and behind the client's back, forcing the client up on their tiptoes. MHT #1 was seen leaning on Client #3 with her upper body into the client's upper body area pressing Client #3's chest against the wall. Client #3 was seen having to turn their head to the side with cheek against the wall. This hold was maintained for 23 seconds then MHT #1 repositioned her arms and continued to hold the client's arms behind their back, Client #3 remained against the wall and on their tiptoes. Client #3 and MHT #1 were seen having a verbal exchange, but the video did not have audio. At 7:13:26 PM, MHT #2 entered the hallway, approached the client, and took over the restraint by placing his arms under the client's armpits and pulling the elbows up and behind the client, lifting the client back up on their tiptoes. MHT #2 interlaced his own fingers together while pulling Client #3's elbows toward each other behind the client's back. MHT #2 leaned his upper body into the back of the client's upper body resulting in the client's chest being pushed against the wall. RN #1 arrived at 7:14:24. MHT #2 readjusted his position standing more upright. MHT #2 stepped back maintaining the physical hold but no longer leaned on Client #3 and lowered the client enough that Client #3's feet were in a flat position on the floor. At 7:16:09 Client #3 was released from the physical restraint. RN #1 looked at Client #3's arm and the client was seen exiting the hallway at 7:16:27 PM. Client #3 was not seen struggling at any time during the physical restraint hold which was referred to as a PRT by the staff. During a telephone interview on 06/04/2025 at 10:23 AM, MHT #1 reported that Client #3 had gone to their room and slammed the door after she had stepped on a bug that Client #3 was	N 128			

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N 128	Continued From page 5 trying to put into a container. MHT #1 reported that she heard small plastic blocks hit the floor. She went into the room and escorted Client #3 out into the hall and placed the client in a PRT. MHT #1 reported that Client #3 was hitting the bookshelves with their fists. MHT #1 reported that the client had told her in the hallway that she had hurt the client 's wrist, but she did not report the complaint to MHT #2 who took over the restraint later in the hallway. MHT #1 stated she did not come in on her next scheduled day, "Because I figured I'd be suspended." During a phone interview on 06/04/2025 at 10:54 AM, MHT #2 reported that on 05/23/2025 he responded to the restraint of Client #3 and relieved MHT #1 because the staff and the client were arguing. Client #3 stated to MHT #1 "I'm going press charges on you." MHT #2 stated that the resident was hurting and crying and when the nurse arrived MHT #2 stated the client was released from the PRT. MHT #2 described the client's wrist as "bent up and purple." During an interview on 06/04/2025 at 3:00 PM, Client #4 reported that Client #3 had been hurt and described the incident stating that Client #3 had got mad when MHT #1 had stomped on a bug that Client #3 had wanted to catch. Client #4 reported that Client #3 had gone into the bedroom and slammed the door and was behind the door when MHT #1 pushed the door open and it hit Client #3 and hurt their arm. Then, MHT #1 had brought Client #3 out into the hall and put them in a PRT. Client #4 stated Client #3's arm was broken. During an interview on 06/04/2025 at 3:30 PM, Client #3 reported that on 05/23/2025, they had a	N 128			

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N 128	Continued From page 6 verbal confrontation with MHT #1 and went to the bedroom and slammed the door after MHT #1 stomped on a bug that the client had wanted to catch. Client #3 stated that MHT #1 came into the bedroom, slammed the door and started screaming and cursing. Client #3 reported turning toward the cubby to get a "fidget" out to "calm myself down. As I turned, she pushed me from behind and my hand hit the cubby and went numb and then she put me in a PRT." Client #3 reported that their hand and wrist bent backwards when it hit the cubby. Client #3 reported that MHT #1 took them out into the hallway and continued the PRT. Client #3 stated "I felt a 'pop' and it felt like it shifted, and the bones rubbed together. At first, it was numb but then she was putting pressure on it, and it started hurting really bad. I told her she had broken it." Client #3 reported that MHT #2 took over the PRT and "put so much pressure on my arm that I was on my toes and then I had a bruise on my arm." Client #3 indicated to this surveyor the upper bicep area as being where the bruise was located. Client reported, "Then the nurse came, and they let me go, on my way to the quiet room my hand was purple." During an interview on 06/04/2025 at 4:16 PM, Client #1 reported witnessing the incident and stated he heard Client #3 in their room with MHT #1 saying, "I didn't do anything, why did you put me in a PRT?" Client #1 stated that Client #3 was saying, "Stop! Stop! Stop! I feel like my arm is broken!" During a phone interview on 06/05/2025 at 7:36 AM, MHT #3 reported being present during the incident on 05/23/2025, and revealed that Client #3 was fighting with MHT #1 as she was bringing	N 128			

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N 128	Continued From page 7 the client out into the hallway and was screaming, "My arm, my arm- you broke my arm!" During an interview on 06/05/2025 at 3:25 PM, the Director of Nursing (DON) reported that unless the client was a danger to self or others, interventions such as offering coping skills, redirection, or removing the audience should be attempted by staff prior to placing a resident in a physical restraint. The DON also reported that staff were taught to call the nurses before putting clients in a physical hold. During an interview on 06/05/2025 at 3:45 PM, the Chief Executive Officer (CEO) stated that staff should not put a resident in a physical restraint unless the resident was a harm to self or others. Staff were to give clients alternatives and try the coping skills that have been identified as effective for them. Some have "fidgets" that help them calm down and when restraints are used it should be the least restrictive method. Review of an " Emergency Department Physician Documentation " dated 05/23/2025, revealed Client #3 had diagnoses of a break in the larger wrist bone, about an inch below the wrist joint and a fracture of both radial and ulnar bones. Review of a facility policy titled, "Behavioral Management-General" revised 03/2025, revealed that restraints are used only in the case of emergency, when there is imminent risk of harm to the client or others and when non-physical interventions would or have been ineffective . "These techniques are used only after less restrictive methods have been tried and failed and all other means of verbal de-escalation techniques have been expended." The policy	N 128			

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N 128	Continued From page 8	N 128			
N 142	indicated if restraints are used, they are to be used in the least restrictive manner and are to be discontinued at the earliest time possible. ORDERS FOR USE OF RESTRAINT OR SECLUSION CFR(s): 483.358(c) A physician or other licensed practitioner permitted by the state and the facility to order restraint or seclusion must order the least restrictive emergency safety intervention that is most likely to be effective in resolving the emergency safety situation based on consultation with staff. This ELEMENT is not met as evidenced by: Based on observations, interviews and record review, the facility failed to ensure that the least restrictive interventions were implemented prior to physically restraining one (Client #3) of three case mix clients reviewed for restraints. The findings include: Review of Client #3's "Master Treatment Plan," dated 05/06/2025, revealed Client #3 had diagnoses which included major depressive disorder which was recurrent and severe without psychosis, generalized anxiety disorder, history of post-traumatic stress disorder, and history of trauma. Review of an "Emergency Safety Intervention Justification" packet (ESI) dated 05/23/25, and completed by Registered Nurse (RN) #1, indicated Client #3 was placed in a physical restraint at 7:11 PM, and was released at 7:15	N 142			

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N 142	Continued From page 9 PM, due to "resident out of control" and "uncooperative with staff." ESI documentation included check marks under the section that listed interventions attempted before restraint as: emphasized need for self-control, attempted to redirect, and allowed patient other ways to vent anger. Review of the "Client Debrief Form" contained in the ESI packet revealed Client #3 reported that the staff gave no warnings before initiating restraint. Review of the "Staff Debrief Form" contained in the ESI packet signed by Mental Health Technician (MHT) #1 revealed the measure taken to de-escalate the situation prior to implementing the restraint was, processed with patient. Review of a "Body Check Form" contained in the ESI packet under the comment section dated 05/23/2025 at 7:17 PM, signed by RN #1 indicated that per staff, Client #3 was placed in a personal restraint due to being uncooperative with staff and slamming the bedroom door in another resident's face. Review of a handwritten "Witness Statement," by MHT #1 dated 05/23/2025, located within the facility investigation packet, indicated Client #3 called MHT #1 a derogatory name after she stepped on a bug that Client #3 was trying to put in a "container and save." Client #3 then went into the bedroom and slammed the door and was banging hands on the clothes shelf. MHT #1 reported she told Client #3 to come out of the bedroom, but Client #3 was "resisting staff" but finally came out and was placed in a primary restraint technique (PRT).	N 142			

DEPARTMENT OF HEALTH AND HUMAN SERVICES
CENTERS FOR MEDICARE & MEDICAID SERVICES

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FORM APPROVED
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STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: 04L115	(X2) MULTIPLE CONSTRUCTION A. BUILDING _____ B. WING _____		(X3) DATE SURVEY COMPLETED C 06/06/2025
NAME OF PROVIDER OR SUPPLIER WOODRIDGE OF FORREST CITY, LLC			STREET ADDRESS, CITY, STATE, ZIP CODE 1521 ALBERT ST FORREST CITY, AR 72335		
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N 142	Continued From page 10 Review of a "Witness Statement" transcribed and written by the Quality and Risk Manager due to resident's injury received on 05/23/2025 during an Emergency Safety Intervention and signed by the resident on 05/26/2025, revealed Client #3 reported finding a bug but MHT #1 stepped on it and both Client #3 and MHT #1 began calling each other derogatory names. Client #3 went into the bedroom and slammed the door, then MHT #1 came into the room, slammed the door, screamed "don't slam this [expletive] door!" Client #3 stated, "I won't slam this [expletive] door in a sassy way. " The client reported being shoved into the cubby by MHT #1 and then being put into a PRT and taken out into the hallway. Client #3 stated having asked MHT #1 "could you release the pressure on my left arm because I think you broke it." Review of video footage dated 05/23/2025 time stamped beginning at 7:11:47 PM and ending at 7:16:22 PM, revealed the following events: At 7:11:49 Client #1 was seen noticing something on the floor by the double doors, turned and went quickly back to the bedroom, returned five seconds later with a red container and went back to the door and was seen bending down. MHT #1 arrived at the door and stepped in front of Client #3 and appeared to step on something. Client #3 went back to the room and shut the door at 7:12:15 PM. One second later, MHT #1 was seen aggressively pushing past Client #4 who stood in the doorway. Client #4 continued to look into the open doorway and then the door closed again at 7:12:26 PM. One second later, Client #3 was seen coming out of the room with MHT #1 holding the client's left upper extremity around the bicep	N 142			

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CENTERS FOR MEDICARE & MEDICAID SERVICES

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N 142	Continued From page 11 area with her left hand. MHT #1's arm was hooked under the client's right armpit which resulted in Client #3 ' s right arm being held behind the client's back. MHT #1 then forcefully put Client #3 against the hallway wall, pulled the client's elbows at an approximate 90-degree angle to the client's body, up and behind the client's back, forcing the client up on their tiptoes. MHT #1 was seen leaning on Client #3 with her upper body into the client's upper body area pressing Client #3's chest against the wall and the client was seen having to turn their head to the side with cheek against the wall. This hold was maintained for 23 seconds, then MHT #1 repositioned her arms and continued to hold Client #3's arms behind the client ' s back and the client remained against the wall and on their tiptoes. Client #3 and MHT #1 were seen having a verbal exchange, but the video did not have audio. At 7:13:26 PM, MHT #2 entered the hallway, approached Client #3 and took over the restraint by placing his arms under the client's armpits and pulling the elbows up and behind the client, lifting the client back up on tiptoes. MHT #2 interlaced his fingers together while pulling Client #3's elbows toward each other behind the client's back. MHT #2 was leaning his upper body into the back of the client's upper body resulting in Client #3 ' s chest being pushed against the wall. RN #1 arrived and at 7:14:24 PM, MHT #2 readjusted his position stood more upright, stepped back to maintain the physical hold, but no longer leaned on Client #3 and lowered Client #3 enough that the client's feet were in a flat position on the floor. At 7:16:09 PM, Client #3 was released from the physical restraint, RN #1 looked at Client #3's arm and client was seen exiting the hallway at 7:16:27 PM. Client #3 was not seen struggling at any time during the	N 142			

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CENTERS FOR MEDICARE & MEDICAID SERVICES

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N 142	Continued From page 12 physical restraint hold which was referred to as a PRT by the staff. On 06/04/2025 during a telephone interview which began at 10:23 AM, MHT #1 reported that Client #3 had gone to the room and slammed the door after she had stepped on a bug that the client was trying to put into a container. MHT #1 reported that she heard small plastic blocks hit the floor, and she went into the room and escorted Client #3 out and placed the client in a PRT. On 06/04/2025 during a phone interview which began at 10:54 AM, MHT#2 reported that he had responded on 05/23/2025, to the restraint of Client #3 that was already in progress. He had relieved MHT #1 because the staff and the client were arguing and Client #3 had stated to MHT #1 "I'm gonna press charges on you." MHT #2 stated that the resident was hurting and crying and when the nurse arrived, the client was released from the PRT. MHT #2 described Client #3's wrist as "bent up and purple." During an interview on 06/04/2025 at 3:00 PM, Client #4 reported that Client #3 had been hurt by staff and described the incident. Client #4 stated Client #3 had gotten mad when MHT #1 had stomped on a bug that Client #3 had wanted to catch. Client #4 reported that Client #3 had gone into the bedroom and slammed the door and was behind the door, then MHT #1 had pushed the door open, and it hit Client #3 and hurt the client's arm. Then MHT #1 had brought Client #3 out and put them in a PRT. Client #4 stated Client #3's arm was broken. During an interview on 06/04/2025 at 3:30 PM,	N 142			

DEPARTMENT OF HEALTH AND HUMAN SERVICES
CENTERS FOR MEDICARE & MEDICAID SERVICES

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N 142	Continued From page 13 Client #3 reported that on 05/23/2025, they had a verbal confrontation with MHT #1 and went to the bedroom and slammed the door after MHT #1 stomped on a bug that the client had wanted to catch. Client #3 stated MHT #1 came into the bedroom slammed the door and started screaming and cursing. Client #3 reported turning toward the cubby to get a fidget out to "calm myself down, as I turned, she pushed me from behind, my hand hit my cubby, bent backwards and went numb, and then she put me in a PRT." Client #3 reported that MHT #1 took them out to the hallway and continued the PRT in the hall where the client felt a "pop" and stated it "felt like it shifted, and the bones rubbed together. At first it was numb, but then she put pressure on it, and it started hurting really bad. I told her she had broken it." Client #3 reported that MHT #2 took over the PRT and "put so much pressure I was on my toes and had a bruise on my arm." Client #3 then indicated to this surveyor the upper biceps area as being where the bruise was located. Client #3 stated, "Then the nurse came, and they let me go; on my way to the quiet room my hand was purple. " During an interview on 06/04/2025 at 4:16 PM, Client #1 reported witnessing the incident. Client #1 stated they heard Client #3 in the room with MHT #1 saying, "I didn't do anything why did you put me in a PRT?" Client #1 stated that Client #3 was saying, "Stop! Stop! Stop! I feel that my arm is broken!" On 06/05/2025 during a phone interview which began at 7:36 AM, MHT #3 reported being present during the incident on 05/23/2025. MHT #3 stated Client #3 was fighting with MHT #1 as she was bringing the client out into the hallway.	N 142			

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CENTERS FOR MEDICARE & MEDICAID SERVICES

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N 142	Continued From page 14 Client #3 was screaming, "My arm- you broke my arm!" During an interview on 06/05/2025 at 1:29 PM, RN #2 reported that staff were taught to give at least three verbal prompts and try to de-escalate the behavior before a client was placed in a physical restraint. During an interview on 06/05/2025 at 3:25 PM, the Director of Nursing (DON) reported that unless the client was a danger to self or others, interventions such as offering coping skills, redirection, or removing the audience should be attempted by staff prior to placing a resident in a physical restraint. The DON stated that staff were taught to call the nurses before putting clients in a physical hold. During an interview on 06/05/2025 at 3:45 PM, the Chief Executive Officer (CEO) stated that staff should not put a resident in a physical restraint unless the resident was a harm to self or others. The CEO stated staff gave clients alternatives and the coping skills that have been identified to work for them. Some have "fidgets" that help them calm down and when a restraint was used it should be the least restrictive method. Review of a facility policy titled, "Behavioral Management-General" with a revision date of 03/2025, revealed that restraint is used only in the case of emergency, when there is imminent risk of harm to the resident or others and when non-physical interventions would or have been ineffective. These techniques are used only after less restrictive methods of interventions have been tried and failed and all other means of	N 142			

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N 142	Continued From page 15 verbal de-escalation have been expended. If restraint is used it is to be used in the least restrictive manner possible and discontinued at the earliest possible time.	N 142	The Resident affected was assessed by Nursing and an Order was received by the Physician for further evaluation of Resident at local Medical center. Staff involved were suspended immediately and remain suspended pending investigation results. Results from internal investigation yield one Staff member not returning. The likelihood for this deficiency affecting other Residents being affected was greatly reduced and minimized due to the organization's response. The Chief Executive Officer oversees the implementation of this corrective action plan. A mandated facility-wide retraining for all Mental Health Technicians, Nurses, Dietary Staff, Maintenance Staff, Clinical staff, and Administrative staff on verbal de-escalation and proper physical interventions was conducted by the Handle with Care Instructor June 18th, 2025. Retraining included modeling verbal de-escalation, proper physical intervention, and scenario-based participation. All attendees signed acknowledging receipt of training and attestation of understanding. and attestation of understanding. For Staff who could not attend the training, individual training will be provided, documented, and attestations will be signed. All training will be completed by July 6th, 2025. The Director of Quality and Risk Management will be the custodian of the training log. Training records will be audited weekly by the Quality Risk Director until 100% training compliance is reached for existing staff. This training is already part of the new hire orientation process. The Director of Quality and Risk Management will continue to review and document all physical interventions with special emphasis on physical interventions where a Resident injury was reported. Observations will be reviewed with the interdisciplinary Safety committee daily (Monday – Friday) comprised of the Director of Nursing, Program Director, Handle with Care Instructor, Clinical Director, Director of Quality and Risk Management, and Chief Executive Officer. Proper reporting to Guardians, Abuse/Neglect hotline, and other child protective entities will continue to be notified in accordance with Arkansas and Resident's corresponding State guidelines. Cumulative data will be reviewed in the Committee of the Whole monthly and reported to the Governing Board for oversight.		



Division of Provider Services
& Quality Assurance
P.O. Box 8059, Slot S404
Little Rock, AR 72203-8059

June 30, 2025

Charlotte Lockhart, Administrator
Woodridge Of Forrest City, Llc
1521 Albert St
Forrest City, AR 72335

Dear Administrator:

On June 6, 2025, we conducted a Complaint Investigation survey at your facility. You have alleged that the deficiencies cited on that survey have been corrected. We are accepting your allegation of compliance and have approved your plan of correction and presume that you will achieve substantial correction by July 6, 2025.

We will be conducting a revisit of your facility to verify that substantial correction has been achieved and maintained.

If you have any questions, please contact your reviewer: **Breanna Marengo at 501-320-6280 or email to: breanna.marengo@dhs.arkansas.gov.**

Sincerely,

A handwritten signature in black ink, appearing to read "B. Marengo".

for Jeff Rosenbaum

Breanna Marengo, RN QA Reviewer
DPSQA/Office of Long Term Care
Survey & Certification Section

bbm